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Terminal Illness Claims

Health Claims Forum – 16 May 2013

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Agenda



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- Terminal Illness Claims Study – Introduction
- Study Results – Brendon Gerding, Senior Actuarial Analyst, PLRe
- Practical Claim Management – Paul Reddick, Head of Medical Underwriting & Claims, PLRe
 - Medical Factors
 - Evidence gathering
 - PLRe Philosophy
 - Conclusions
- Cancer, Palliative Treatment and Survival Rates - Dr James D. Brenton
PhD FRCP

The Study – Summary

- We analysed around 2,700 terminal illness claims that were paid under life insurance policies over a period of several years.
- The objective was to determine whether the claimant was alive or dead
- The data is from several companies so reflects a range of claim philosophies

Why undertake a study?

- A unique opportunity to evaluate the accuracy and impact of Terminal illness claim decisions - over a long time period
- To identify and evaluate any patterns or trends
- To see if there are any circumstances where faster claim payments can be made (e.g. Specific cancer sites)



- Terminal Illness claims as a proportion of all death claims have increased over the last 5 years - now account for 25% of all claims
- Payments are based on the likelihood of death occurring within a relatively short time-frame - we want to understand if we are getting it right
- This is a highly sensitive area - could we identify any opportunities to remove obstacles at this difficult time by making faster claims decisions - without compromising accuracy?
- Would there be any cancer diagnoses or other conditions where survival was materially longer than might have been expected?



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Practical Claim Management

Paul Reddick

Head of Medical Underwriting & Claims

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- Primary Factors
 - Evidence gathering
- PLRe Philosophy
 - Conclusions

TI claims by primary tumour site

	Site	Co. A	Co. B	Co. C	Average
1	Lung	24.7%	19.0%	23.2%	21.0%
2	Digestive	13.6%	10.9%	11.0%	11.4%
3	Brain	10.0%	9.6%	10.9%	10.0%
4	Bowel	8.4%	10.2%	8.9%	9.6%
5	Breast	5.4%	9.2%	5.4%	7.6%
6	Urological	7.7%	8.2%	5.7%	7.5%
7	Pancreatic	6.1%	6.7%	5.7%	6.3%
8	Haematological	2.5%	3.3%	3.6%	3.2%
9	Gynaecological	1.7%	3.6%	3.2%	3.2%
10	Other Cancers	14.9%	15.2%	19.6%	16.2%
11	Non Cancers	5.0%	4.1%	2.8%	3.9%

TI claim management – Primary factors



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What are the primary factors in determining life expectancy for advanced cancer?

- Diagnosis
- Age
- Patient Performance status
- Treatment

What are the primary factors in determining life expectancy for advanced cancer?

- Diagnosis
- Site of the primary tumour
- Stage (Stage/Grade, TNM or other classification)
- Extent of lymph node disease (local or distant)
- Extent of metastatic disease – single or multiple sites
 - Metastatic disease is usually incurable but there are exceptions
 - Wide variance in expectations, depending on the location and extent of the metastatic disease
 - Not in itself a reliable indicator for life expectancy of less than 1 year



Prostate Cancer

5-year relative survival by stage at the time of diagnosis

<u>Stage</u>	<u>5-year relative survival rate</u>
Local	nearly 100%
Regional	nearly 100%
Distant	28%

- **Local stage** means that there is no sign that the cancer has spread outside of the prostate.
- **Regional stage** means the cancer has spread from the prostate to nearby areas. This **includes** stage III cancers and **stage IV cancers that haven't spread to distant parts of the body, such as T4 tumours and cancers that have spread to nearby lymph nodes (N1).**
- **Distant stage** includes the rest of the stage IV cancers – all cancers that have spread to distant lymph nodes, bones, or other organs (M1).



What are the primary factors in determining life expectancy for advanced cancer?

- Age
- **Generally, we would expect to see some improvement in life expectancy in younger people.....**

5-Year Relative Survival Rate -

	Age		
	20-44	45-54	55-64
Anaplastic astrocytoma	49%	31%	9%
Glioblastoma Multiforme	17%	6%	4%

What are the primary factors in determining life expectancy for advanced cancer?

- Performance Status
- *The single most important predictive factor in cancer is Performance Status ('functional ability,' 'functional status'): a measure of how much a patient can do for themselves, their activity and energy level. (Dr. David E Weissman MD (End of Life/Palliative Education Resource Center EPERC – Medical College of Wisconsin)*
- The Performance Status will be assessed for advanced stage cancer patients
- An important indicator that directly influences what treatment will be available
- Will have a direct bearing on the patient life expectancy
- Correlation with median survival – e.g. A median survival of 3 months roughly correlates with a Karnofsky score <40 or WHO/ECOG > 3.

Performance Status - WHO/ECOG (Zubrod Score)



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Introduced in 1960 (ECOG – Eastern Co-operative Oncology Group)

Grade	Explanation of activity
0	Asymptomatic – (Fully active, able to carry on all pre-disease performance without restriction)
1	Symptomatic but completely ambulatory – (Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature, e.g., light house work, office work)
2	Symptomatic - <50% in bed during the day (Ambulatory and capable of all self-care but unable to carry out any work activities. Up and about more than 50% of waking hours)
3	Symptomatic - >50% in bed but not bedbound (Capable of only limited self-care, confined to bed or chair more than 50% of waking hours)
4	Bedbound – (Completely disabled. Cannot carry on any self-care. Totally confined to bed or chair)

Performance Status – Karnofsky Score



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Developed in 1948

- 100% – normal, no complaints, no signs of disease (WHO 0)
- 90% – capable of normal activity, few symptoms or signs of disease (WHO 0)
- 80% – normal activity with some difficulty, some symptoms or signs (WHO 1)
- 70% – caring for self, not capable of normal activity or work (WHO 2)
- 60% – requiring some help, can take care of most personal requirements (WHO 2)
- 50% – requires help often, requires frequent medical care (WHO 3)
- 40% – disabled, requires special care and help (WHO 4)
- 30% – severely disabled, hospital admission indicated but no risk of death
- 20% – very ill, urgently requiring admission, requires supportive measures or treatment
- 10% – moribund, rapidly progressive fatal disease processes



- Can the tumour be (completely) resected?
- Has treatment commenced, how many cycles of treatment are planned and has the claimant's response to treatment been assessed?
- Is there evidence of progression following first line chemotherapy?
- Is prolonged survival a realistic treatment aim or is symptom control the only expectation?
- Is the term 'palliative' a decisive factor for claim assessors in determining whether the terminal illness definition has been met?
- Do we know, based on their treatment what the 'optimum' outcome is for the patient?

Possible area of improvement for claims teams?

- Do we really understand the exact nature of modern chemotherapy treatments and their expected effects?
- How should we consider cases with experimental treatments?
- Evolution of treatment – Targeted therapies, Immunotherapy
- Common reason for CMO referral?

Useful Link!

<http://www.cancer.org/treatment/treatmentsandsideeffects/treatmenttypes/chemotherapy/chemotherapy-principlesanin-depthdiscussionofthetechniquesanditsroleintreatment/chemotherapy-principles-types-of-chemo-drugs>

How good are our claim forms?

- Grading/staging (at diagnosis)
- Size, extent and location of spread (how detailed is this?)
- Current treatment
- Previous treatment and response
- Further treatment plans
- Current staging (disease progression?)
- When did life expectancy become less than 12 months?
- **Median survival** or patient specific?
- Has the patient been advised?

Evidence gathering

Company	Request additional information
Company A	Approximately 10%
Company B	5% - 10%
Company C	10%
Company D	0%
Company E	About 10% - 15%

Review of TI claims block

- Vast majority used median survival information
- Oncologist reports occasionally obtained (circa 30%)
- Details of nodal and/or metastatic disease patchy
- No details of patient performance status
- Relatively high proportion required CMO referral



- *“Median survival 10-12 months”*
 - At diagnosis?
 - During or post treatment?

- 50/50 chance the patient will be alive at 12 months
 - Which side of the 12 months line will your claimant fall?
 - After 12 months, life expectancy will vary dramatically depending on a range of factors (e.g. site - lung cancer versus breast cancer & performance status) – Are these factors given due consideration?

OBJECTIVES

- To obtain the optimum evidence, based on the current circumstances of the individual
- To avoid having to go back for additional information or clarification
- To avoid reliance on generic information such as median survival statistics
- To make sure we have the most relevant facts for CMO review
- To reach the correct decision as quickly as possible

Specialist claim form - key information



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DIAGNOSIS

- Histology
- TNM staging
- Any other staging or measurement method e.g. Gleason score, Dukes stage etc.
- Lymph node involvement
- Metastatic spread (site, number and size)

TREATMENT

- Chemotherapy inc dates
- Surgery (is metastatic disease resectable?)
- Further available treatment options
- Optimum expected outcome from available treatment (extended life expectancy, symptom control etc.)

CURRENT STATUS

- ~~Median Survival~~
- Results of most recent scans - evidence of remission/progression?
- Response to treatment
- Performance Status
- Maximum life expectancy for this individual?

Capturing the data



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Details of Metastatic Disease

Location	Number of tumours	Resectable?
Liver	2	No
Lung	1	No

Capturing the data

Please indicate the current performance status

WHO/ECOG

0		1		2	X	3		4		5			
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How good are our claim forms?

- Should a question about Performance Status be added?
- What is the **maximum** predicted life expectancy for the claimant?
- Remove (or amend) reference to median survival?
- Should we routinely obtain Oncology reports?

Pacific Life Re claims philosophy



Obtaining the appropriate (optimum) evidence at the first stage is key
– enables faster payment of claim

Our primary assessment is based on the individual circumstances of the claimant, not median survival statistics

We try to be flexible on the 12 month contract period

Are we delaying the inevitable?

Conclusions



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- The data supports that insurers are taking a fair approach when assessing TI claims – circa 30% alive at 12 months and circa 15% alive at 24 months
- Using median survival information rather than patient specific information may contribute to those living >12 months
- The questions we ask on our claim forms could be improved thus resulting in more accurate and faster decisions being made

Conclusions



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- Conditions where we should continue as currently include:
 - Lowest survivorship (1yr): Digestive, Pancreatic
 - Lowest survivorship (2yr): Lung, Digestive, Brain, Bowel, Pancreatic
- Conditions where we might be more cautious include:
 - Highest survivorship (1yr): Brain, Breast, Non cancers
 - Highest survivorship (2yr): Urological, Breast, Haematological, Non cancers
- Take care on non-cancer claims (MND)



Questions?